

POSTOPERATIVE



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- PATIENTS IMMEDIATE RECOVERY PERIOD IN PACU
- PACU IS POST-ANESTHESIA CARE UNIT
- ANESTHESIOLOGIST, OR NURSE AND PACU NURSE WORK TOGETHER
- DIFFERENT PHASES OF POSTOP CARE (PHASE I, II, EXTENDED OBSERVATION)
- PATIENT'S SAFETY IS THE PRIMARY DETERMINING FACTOR OR WHERE AND WHAT LEVEL POSTOP CARE IS PROVIDED

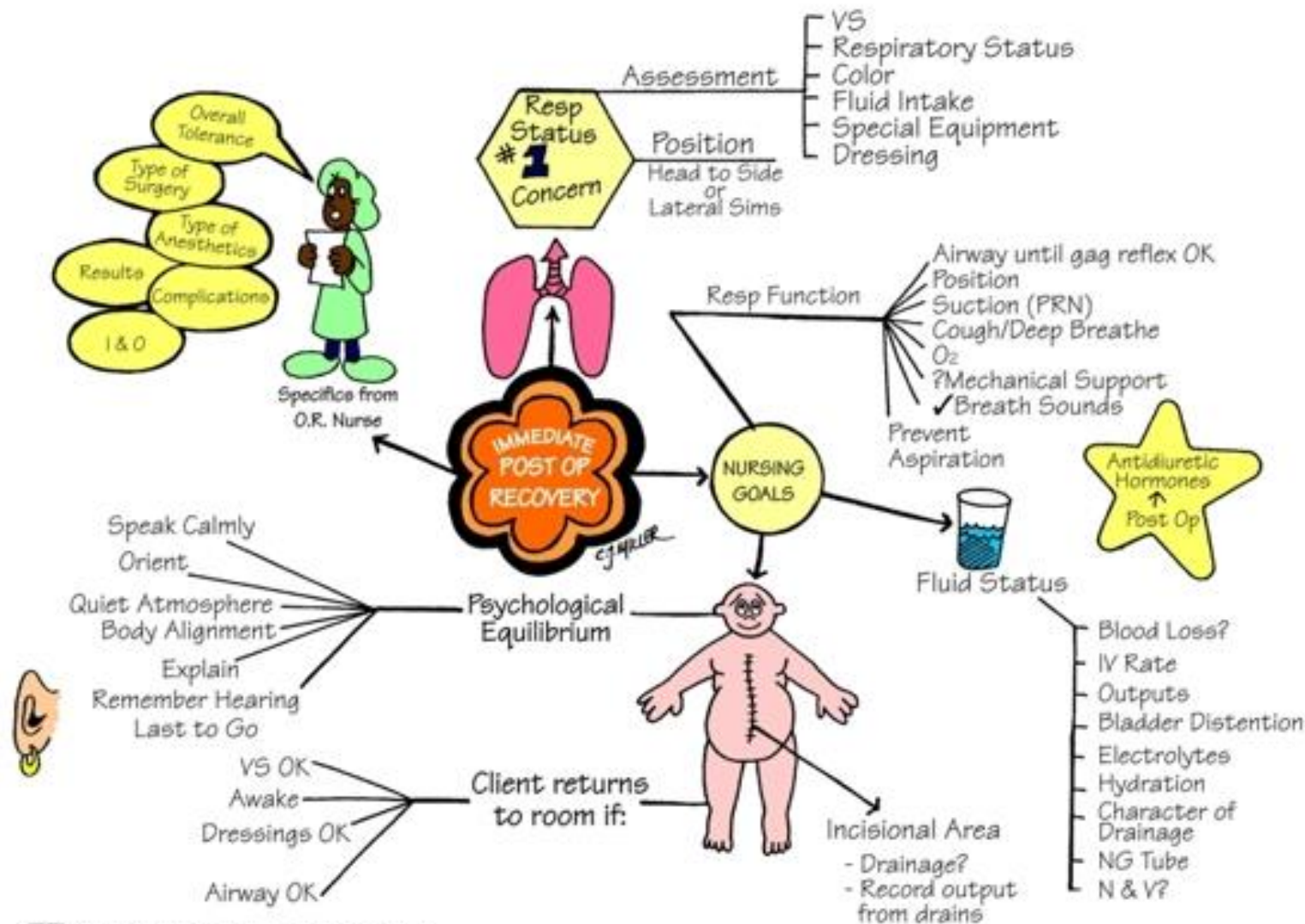
POSTOP: PHASE I INITIAL ASSESSMENT

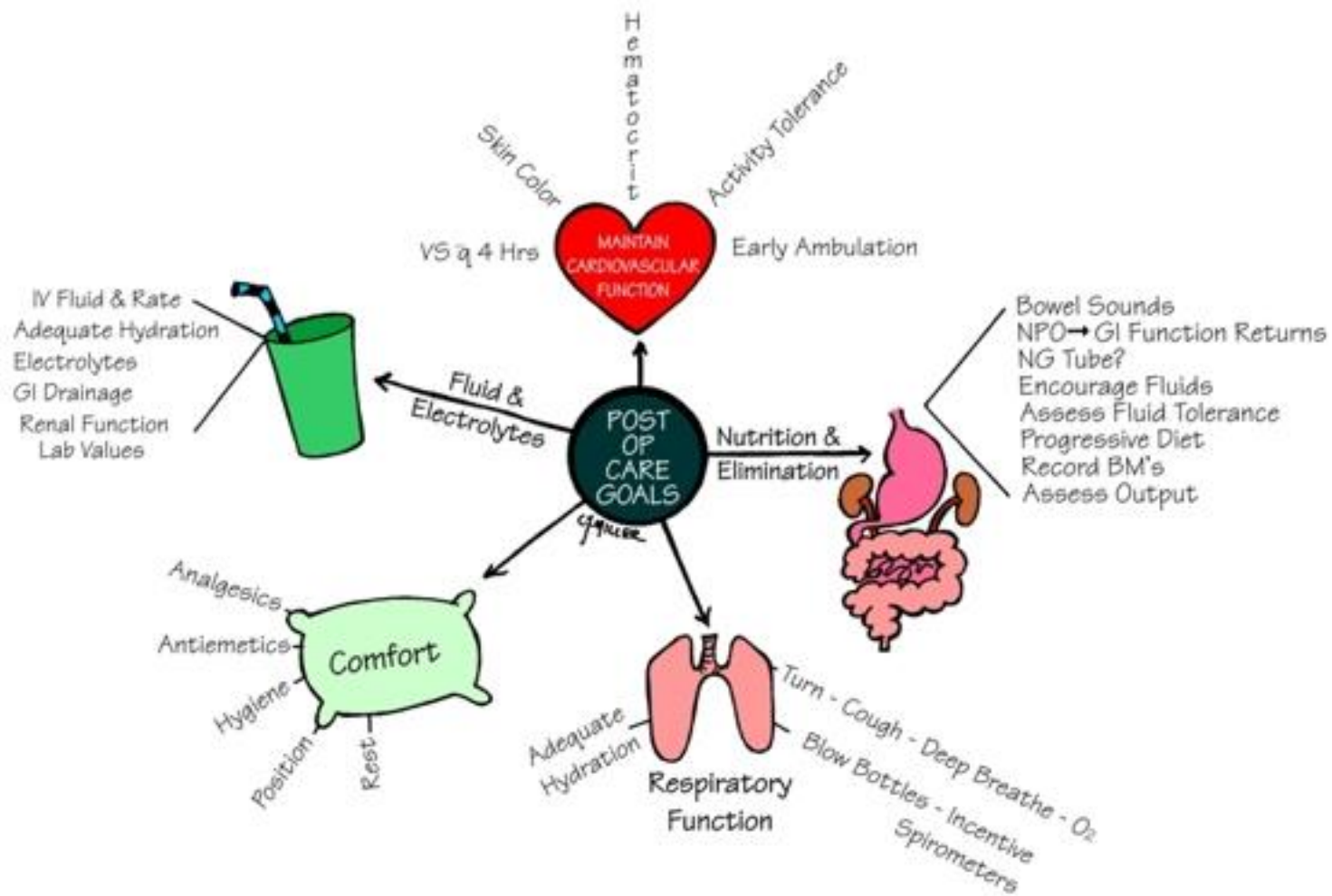
- ANESTHESIOLOGIST/CIRCULATING OR NURSE GIVES A POST ANESTHESIA HAND-OFF REPORT
- HAND-OFF REPORT SHOULD BE STANDARDIZED AND INTERACTIVE (INCLUDE GENERAL INFORMATION, PATIENT HISTORY, INTRAOPERATIVE MANAGEMENT AND COURSE)
- INITIAL ASSESSMENT SHOULD INCLUDE:
 - **AIRWAY** (PATENCY, ENDOTRACHEAL TUBE, VENTILATOR)
 - **BREATHING** (BREATH SOUNDS, RESPIRATORY RATE, O2 %, PAO2)
 - **CIRCULATION** (HEART SOUNDS, BP, HEART RATE, PULSES, EKG, CAP. REFILL, SKIN COLOR, PATENT IV)
 - **GENITOURINARY** (URINE OUTPUT; INTAKE AND OUTPUT)
 - **NEUROLOGICAL** (LOC, ORIENTATION, SENSORY/MOTOR STATUS, PERRLA)
 - **GASTROINTESTINAL** (NAUSEA, VOMITING, BOWEL SOUNDS)
 - **SURGICAL SITE** (INCISION, DRESSING, DRAINAGE, DRAINS)
 - **PAIN** (INCISION AND OTHER PAIN)
 - **PATIENT SAFETY NEEDS** (POSITIONING, FALL RISK)

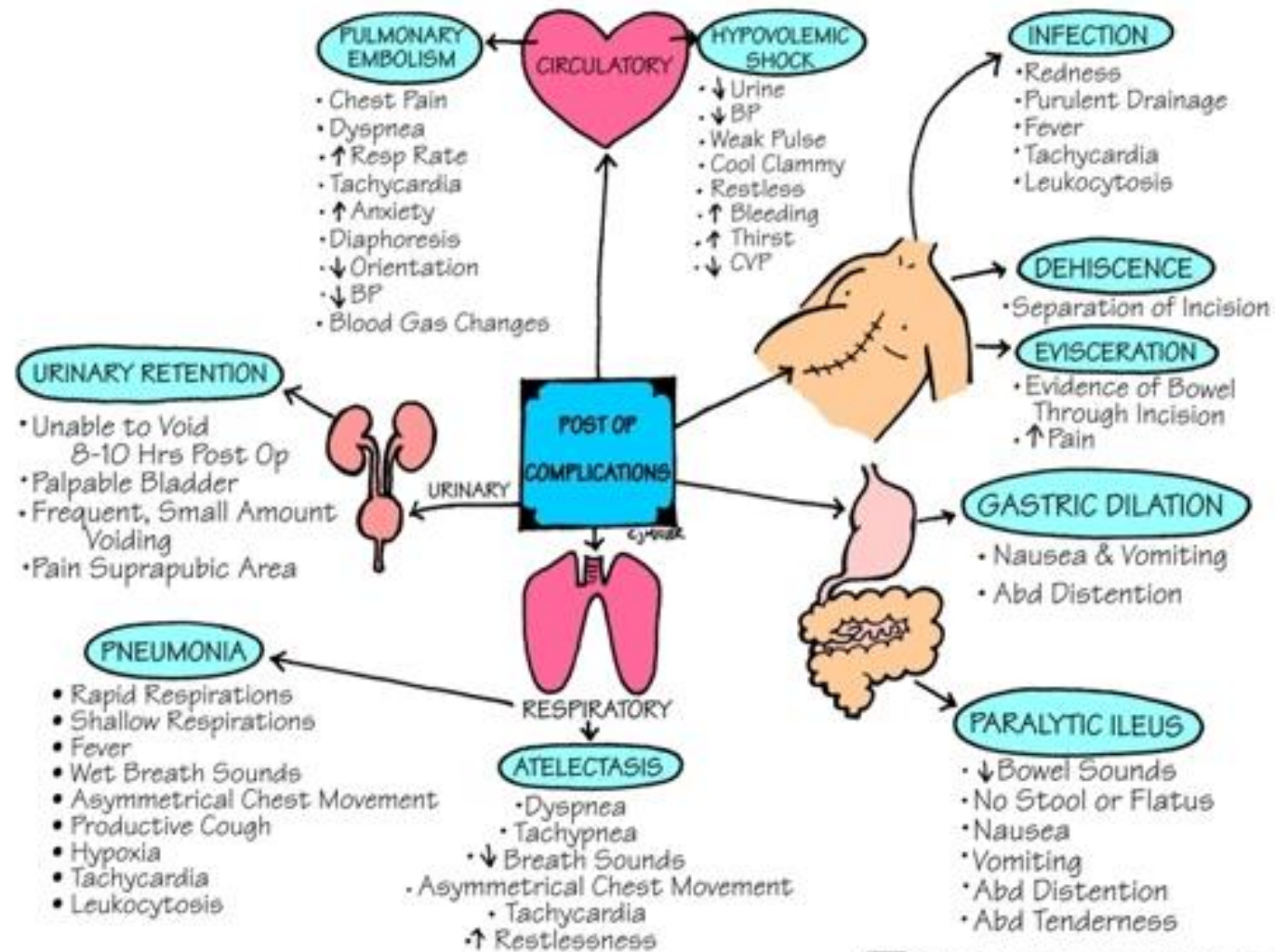
POSTOP: DISCHARGE FROM PACU

- **DISCHARGE FROM PACU**

- LOOK ON TABLE 19-8 IN YOUR MEDICAL SURGICAL TEXTBOOK
- CRITERIA CAN INCLUDE PATIENT AWAKE OR EASILY AROUSABLE, PULSE OX GREATER THAN 90%, VITAL SIGNS STABLE, ETC.
- WHEN DISCHARGING TO CLINICAL UNIT NEED TO PROVIDE
 - HAND OFF REPORT (OPERATIVE AND POSTOPERATIVE)
 - SBAR FORMAT
 - NURSE WHO RECEIVES PATIENT FROM PACU SHOULD SAFELY TRANSPORT PATIENT TO BED, ASSESS AND PROTECT IV LINES AND DRAINS, TAKE VITAL SIGNS AND COMPARE WITH PACU REPORT, FOCUSED ASSESSMENT AND INITIATE POSTOP ORDERS







POSTOP: NEUROLOGICAL PROBLEMS

- **EMERGENCE** DELIRIUM (SHORT-TERM NEUROLOGICAL ALTERATION)
 - RESTLESSNESS, AGITATION, DISORIENTATION, THRASHING, SHOUTING)
 - MAY BE CAUSED BY HYPOXIA, ANESTHESIA, PAIN, BLADDER DISTENSION, ETC.
 - IF DELIRIUM OCCURS, FIRST SUSPECT HYPOXIA
- **DELAYED EMERGENCE**
 - MOST COMMON CAUSE IS PROLONGED DRUG ACTION (OPIOIDS, SEDATIVES, INHALED ANESTHETICS, ETC.)
- **ALCOHOL WITHDRAWAL DELIRIUM**
 - RESTLESSNESS, INSOMNIA, NIGHTMARE, IRRITABILITY, ANXIETY, AUDITORY AND VISUAL HALLUCINATIONS (ATIVAN, BANANA BAG, OR ALCOHOL)

POSTOP: NEURO ASSESSMENT/DIAGNOSIS

- **ASSESSMENT**

- LOC, ORIENTATION, MEMORY, AND ABILITY TO FOLLOW COMMANDS, PERRLA, SLEEP CYCLE, SENSORY AND MOTOR STATUS
- DETERMINE POSSIBLE CAUSE OF MENTAL ALTERNATION

- **DIAGNOSIS**

- ACUTE CONFUSION RELATED TO HYPOXIA, POSTOPERATIVE COGNITIVE FUNCTION, DELIRIUM
- DISTURBED SLEEP PATTERN RELATED TO PAIN, HOSPITAL ENVIRONMENT

- **INTERVENTION**

- MOST COMMON CAUSE OF POSTOP AGITATION IN THE PACU IS HYPOXEMIA
- IF YOU RULE OUT HYPOXEMIA OR OTHER CAUSE OF POSTOPERATIVE DELIRIUM, SEDATION MAY BE HELPFUL IN CONTROLLING AGITATION
- DELAYS IN AWAKENING GENERALLY RESOLVE IN TIME
- NARCAN CAN BE USED IF NECESSARY
- PATIENT SAFETY IS A PRIORITY
- PREVENT/MANAGE POSTOPERATIVE DELIRIUM BY ACHIEVING FLUID AND ELECTROLYTE BALANCE, ADEQUATE NUTRITION, PAIN MANAGEMENT, ADEQUATE SLEEP, ETC.

POSTOP: ALTERATIONS IN TEMPERATURE

- **FEVER**

- CAN ALSO HAVE A FEVER RELATED TO LUNG CONGESTION (INCENTIVE SPIROMETER CAN HELP)
- PATIENT COULD HAVE LOW GRADE FEVER THE FIRST 48 HOURS AFTER SURGERY RELATED TO INFLAMMATION PROCESS OR STRESS FROM SURGERY; HEALING PROCESS (INCREASE IN METABOLISM)
- FEVER USUALLY AFTER 48 HOURS AFTER SURGERY MAY INDICATE INFECTION (SURGICAL SITE, CAUTI, PNEUMONIA OR DVT)
- MALIGNANT HYPERTHERMIA (FAST RISE IN BODY TEMPERATURE AND SEVERE MUSCLES CONTRACTION, TACHYCARDIA, MUSCLE OR JAW BECOME STIFF) IS A RARE, INHERITED (GENETIC) DISORDER THAT IS POTENTIALLY FATAL

- **HYPOTHERMIA** (FROM BODY HEAT LOSS, ANESTHESIA)

- INFANTS (CAN LOSE HEAT 4X FASTER THAN ADULTS), OLDER ADULTS, WOMEN
- LEADS TO VASOCONSTRICTION WHICH CAN LEAD TO HYPERTENSION, CARDIAC PROBLEMS (MI), DECREASED IMMUNE RESPONSE, REDUCED ORGAN BLOOD FLOW
- ANESTHESIA CAN DROP TEMPERATURE AND OR ROOM IS KEPT COLD TO HELP PREVENT BACTERIA GROWTH/KEEP STAFF COMFORTABLE
- SHIVERING CAN CAUSE INCREASE IN O2 DEMANDS AND INCREASE IN HR

POSTOP: TEMP. ASSESSMENT/INTERVENTION

- MEASURE AND RECORD TEMPERATURE
- MEASURE TEMPERATURE EVERY 15 MINS UNTIL NORMAL
- ASSESS VITAL SIGNS
- SIGNS AND SYMPTOMS OF HYPOTHERMIA
 - SHIVERING, EXHAUSTION, CONFUSION, FUMBLING HANDS, MEMORY LOSS, SLURRED SPEECH DROWSINESS
 - INFANTS: BRIGHT RED, COLD SKIN, VERY LOW ENERGY
- O2 THERAPY
- DEMEROL CAN TREAT SHIVERING
- HYPOTHERMIA (WARM BLANKETS/AIR WARMERS, WARM IV FLUIDS/HEATER WATER MATTRESS)-MONITOR SKIN TO PREVENT BURNS OR SKIN INJURIES
- SUPPLIES SHOULD BE READILY AVAILABLE FOR MALIGNANT HYPERTHERMIA (MH)
- MH: ICE PACKS/COOLING BLANKET/COLD IV & ORAL SOLUTIONS/DANTRIUM

POSTOP: PAIN

- PATIENT'S OWN REPORT OF PAIN IS THE MOST RELIABLE INDICATOR OF THEIR PAIN
 - IF PATIENT IS NON VERBAL THEN ASSESS BODY LANGUAGE FOR SIGNS OF PAIN
- ASSESS PATIENT'S PAIN LEVEL AND AN ACCEPTABLE PAIN LEVEL TO ACHIEVE PAIN MANAGEMENT
- ASK FOR THE LOCATION OF PAIN, INTENSITY, QUALITY , ACUTE OR CHRONIC, INTERMITTENT OR CONSTANT
- INTERVENTIONS:
 - PAIN MEDS: OPIOIDS, NSAIDS, PCA PUMP, LOCAL ANESTHESIA
 - CHECK RESPIRATIONS, LEVEL OF CONSCIOUSNESS AND VITAL SIGNS
 - NON-PHARMACOLOGICAL: MASSAGE, MUSIC THERAPY, ETC.

POSTOP: AMBULATORY SURGERY

- WHEN DISCHARGING FROM AMBULATORY SURGERY SETTINGS, PATIENT MUST:
- BE MOBILE AND ALERT TO PROVIDE SELF-CARE WHEN AT HOME
- MUST BE STABLE AND CLOSE TO BASELINE LEVEL OF PREOPERATIVE FUNCTIONING
- PROVIDE DISCHARGE TEACHING TO PATIENT AND FAMILY
- PATIENT CANNOT DRIVE, MUST HAVE SOMEONE DRIVE THEM HOME
- FOLLOW UP APPOINTMENTS
- DISCHARGE MEDICATION LIST
- VIDEOS ARE HELPFUL FOR DISCHARGE TEACHING

POSTOP: DISCHARGE TEACHING

- BEGINS IN PREOPERATIVE TEACHING
- PATIENT IS PREPARED FOR SELF-CARE
- KNOWS WHAT SYMPTOMS TO REPORT AND WHAT ARE EXPECTED
- MEDICATION REGIMEN/PRESCRIPTIONS/SIDE EFFECTS
- INCISION CARE/DRESSING CHANGES (NO SWIMMING OR SOAKING IN TUB)
- KNOW WHAT ACTIVITIES ARE ALLOWED AND NOT ALLOWED
- DIETARY RESTRICTIONS
- FOLLOW UP APPOINTMENTS
- ANSWER PATIENTS QUESTIONS THOROUGHLY
- ADEQUATE PAIN MANAGEMENT BEFORE BEING DISCHARGE
- INVOLVE CASE MANAGEMENT FOR (FINANCES, LONG-TERM CARE, HOME HEALTH, PT, OT)
- DOCUMENT TEACHING IN PATIENT'S CHART

POSTOP: ELDERLY

- PNEUMONIA IS A COMMON POSTOP COMPLICATION IN OLDER ADULTS
- CARDIAC PROBLEMS DUE TO DECREASE IN VASCULAR FUNCTION CAUSED BY STIFFENING OF VESSELS OR PLAQUE BUILD UP IN VESSELS
- DRUG TOXICITY POTENTIAL PROBLEM
- CHANGES IN MENTAL STATUS IS IMPORTANT TO ASSESS
- PAIN CONTROL CAN BE CHALLENGING DUE FEAR, LACK OF EDUCATION, COGNITIVE DEFICITS, ETC.